

Start of Care Assessment, Adult

CCG: C-1109 (CCG)

MCG Health
Chronic Care
28th Edition

- Assessment
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Assessment

Language

Q1. Patient: Do you speak English?

- Yes
- No [Q2] [Q3]
- Unknown [Q2] [Q3]

Q2. Case Manager: What language does the patient speak?

- Arabic [B1]
- Armenian [B1]
- Cambodian [B1]
- Chinese [B1]
- Croatian [B1]
- Czech [B1]
- Danish [B1]
- Dutch [B1]
- Filipino (Tagalog) [B1]
- Finnish [B1]
- Formosan [B1]
- French [B1]
- French Creole [B1]
- German [B1]
- Greek [B1]
- Hebrew [B1]
- Hindi [B1]
- Hmong [B1]
- Hungarian [B1]
- Italian [B1]
- Japanese [B1]
- Korean [B1]
- Lithuanian [B1]
- Navaho [B1]
- Norwegian [B1]
- Persian [B1]
- Polish [B1]

- Portuguese [B1]
- Punjabi [B1]
- Romanian [B1]
- Russian [B1]
- Serbo-Croatian [B1]
- Slovak [B1]
- Spanish [B1]
- Swedish [B1]
- Thai (Laotian) [B1]
- Turkish [B1]
- Ukrainian [B1]
- Vietnamese [B1]
- Yiddish [B1]
- Other, specify: _____ [B1]
- Unknown [B1]

Q3. Patient: Is there anyone available who can translate?

- Yes, the patient's family or caregiver is available and speaks and understands English. [Q4] [Q5] [B1]
- Yes, the patient's family or caregiver speaks and understands English, but arrangements must be made for them to be available. [Q4] [Q5] [B1]
- No [Q6] [B1] [B2] [P1]
- Unknown [Q6] [B1] [B2] [P1]

Q4. Caregiver: Is the patient OK with you speaking on their behalf?

- Yes [Q5]
- No [Q6] [P1]
- Unknown [Q6] [P1]

Q5. Patient: Are there problems with reading or writing?

- Yes [B1] [P1] Health Literacy  CCG
- No
- Unknown [P1] Health Literacy  CCG

Q6. Patient: Is it OK to use a translation service?

- Yes
- No [B2] [B1] [P1]
- Unknown [B2] [B1] [P1]
- Not applicable

Hearing

Q1. Case Manager: Can the patient hear well enough to engage in activities?

- Yes
- No [B3] [P2]
- Unknown [B3] [P2]

Q2. Patient: Do you have a hearing problem?

- Yes [Q3] [Q5] [B3] [P3]
- No [Q4]
- Unknown [Q4] [P2]

Q3. Patient: What do you use to help your hearing or make up for your hearing loss? (Check all that apply.)

- Hearing aids
- Sign language
- Sign language interpreter
- Telecommunications relay service (TRS) (eg, TTY or TDD)
- Writing notes
- Other, specify: _____
- Unknown [P3]
- Nothing [P3]
- Not applicable

Q4. Patient: Do your family members or friends think you have a problem hearing?

- Yes [Q5] [B3] [P3]
- No
- Unknown [P2]

Q5. Patient: Have you seen an otolaryngologist (a specialist hearing doctor) about your hearing problem?

- Yes
- No [P2]
- Unknown [P2]
- Not applicable

Vision

Q1. Patient: Do you have problems with your vision?

- Yes, specify: _____ [Q2] [Q3] [B4] [P4]
- No
- Unknown [P5]

Q2. Patient: Have you seen an ophthalmologist (a specialist eye doctor) about your vision problem?

- Yes
- No [P5]
- Unknown [P5]

Q3. Patient: What do you use to improve your vision or make up for your vision loss? (Check all that apply.)

- Braille [P4]
- Computer software [P4]
- Glasses or contact lenses [P4]
- Global Positioning System (GPS) for the visually impaired [P4]
- Magnifying lenses [P4]
- Sighted companion [P4]
- White cane or mobility training [P4]
- Other, specify: _____ [P4]
- Unknown [P4]
- Nothing [P5]
- Not applicable

Q4. Patient: Do you have trouble seeing to do daily activities? (Check all that apply.)

- Ambulation [B4] [P4]
- Cooking [B4] [P4]
- Dressing [B4] [P4]
- Driving [B4] [P4]
- In low light [B4] [P4]
- Reading [B4] [P4]
- Shaving [B4] [P4]
- Shopping [B4] [P4]
- Watching TV [B4] [P4]
- Other, specify: _____ [B4] [P4]
- Unknown [P5]
- No

Health Status

Q1. Patient: Major Diagnosis: Please tell me about your current health status. What is your main concern? When was it diagnosed?

- Anemia, date diagnosed: _____
- Anxiety, date diagnosed: _____
- Arthritis, date diagnosed: _____
- Asthma, date diagnosed: _____
- Atrial fibrillation, date diagnosed: _____
- Attention-deficit hyperactivity disorder (ADHD), date diagnosed: _____
- Back pain, date diagnosed: _____
- Bipolar disorder, date diagnosed: _____
- Cancer, date diagnosed: _____
- Cardiac congenital defects, date diagnosed: _____

- Chronic obstructive pulmonary disease (COPD), date diagnosed: _____
- Coronary artery disease, date diagnosed: _____
- Cystic fibrosis, date diagnosed: _____
- Dementia, date diagnosed: _____
- Depression, date diagnosed: _____
- Diabetes, date diagnosed: _____
- Heart failure, date diagnosed: _____
- High blood pressure, date diagnosed: _____
- High cholesterol, date diagnosed: _____
- High-risk pregnancy, date diagnosed: _____
- HIV/AIDS, date diagnosed: _____
- Inflammatory bowel disease, date diagnosed: _____
- Kidney disease, date diagnosed: _____
- Liver disease, date diagnosed: _____
- Multiple sclerosis, date diagnosed: _____
- Obesity, date diagnosed: _____
- Organ transplant, procedure date: _____
- Osteoporosis, date diagnosed: _____
- Parkinson disease, date diagnosed: _____
- Pregnancy, date diagnosed: _____
- Schizophrenia, date diagnosed: _____
- Seizure disorder, date diagnosed: _____
- Sickle cell disease, date diagnosed: _____
- Sleep apnea, date diagnosed: _____
- Spinal cord injury, date diagnosed: _____
- Stroke, date diagnosed: _____
- Substance use disorder, date diagnosed: _____
- Traumatic brain injury, date diagnosed: _____
- Other, specify: _____
- Unknown

Q2. Patient: Comorbid Conditions: What other health problems do you have? (Check all that apply.)

- Anemia
- Anxiety
- Arthritis
- Asthma
- Atrial fibrillation
- Attention-deficit hyperactivity disorder (ADHD)
- Bipolar disorder
- Blood clot
- Cancer
- Cardiac congenital defects
- Chronic obstructive pulmonary disease (COPD)
- Coronary artery disease
- Cystic fibrosis
- Dementia
- Depression
- Diabetes
- Heart failure
- High blood pressure
- Infections or immune system problems
- Kidney failure
- Liver disease
- Obesity or overweight
- Osteoporosis
- Peripheral vascular disease
- Seizures
- Sleep apnea
- Stroke or other neurologic problem
- Substance use disorder
- Traumatic brain injury
- Wounds (from surgery, injury, or pressure)
- Other, specify: _____

- None

Q3. Patient: Providers: What are the names, specialties, last appointment date, and next scheduled appointment date of the providers you see?

- Provider, specialty, date of last appointment, next scheduled appointment: _____
- Provider, specialty, date of last appointment, next scheduled appointment: _____
- Provider, specialty, date of last appointment, next scheduled appointment: _____
- Provider, specialty, date of last appointment, next scheduled appointment: _____
- Provider, specialty, date of last appointment, next scheduled appointment: _____
- Provider, specialty, date of last appointment, next scheduled appointment: _____
- Provider, specialty, date of last appointment, next scheduled appointment: _____

Q4. Patient: Treatment History: Please describe your treatment for your main health problem. I'm interested in medicines, hospital stays, procedures, and other treatments.

- Treatment history: _____

Q5. Patient: Hospital Inpatient Stays: Have you ever stayed overnight in a hospital? If so, when and for what reason?

- Hospital inpatient stays: _____ [Q6] Hospitalization Risk Assessment [CCG](#)

Q6. Patient: Have you been to the emergency room since your last hospitalization?

- Yes Hospitalization Risk Assessment [CCG](#)
- No
- Unknown

Q7. Patient: Do you seek healthcare from anyone other than your doctor (such as healers, herbalists, or acupuncturists)?

- Yes, specify: _____ Cultural Practices [CCG](#)
- No
- Unknown

Q8. Patient: Do you have any medical allergies (such as to medicines or latex)?

- Yes, specify: _____
- No
- None known
- Unknown

Q9. Patient: What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q10]
- *Haemophilus influenzae* type b (Hib) [Q10] [Q11]
- Hepatitis A [Q10]
- Hepatitis B [Q10]
- Human papillomavirus (HPV) [Q10]
- Influenza [Q10] [Q11]
- Measles, mumps, rubella (MMR) [Q10]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q10]
- Meningococcal B (MenB) [Q10]
- Pneumococcal 13-valent conjugate (PCV13) [Q10] [Q12]
- Pneumococcal 15-valent conjugate (PCV15) [Q10] [Q12]
- Pneumococcal 20-valent conjugate (PCV20) [Q10] [Q12]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q10] [Q12]
- Polio (inactivated) [Q10]
- Respiratory syncytial virus (RSV) [Q10]
- Rotavirus [Q10]
- Tetanus booster [Q10]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q10]
- Varicella [Q10]
- Zoster [Q10]
- Other, specify: _____ [Q10]
- None, because: _____ [P6]
- Unknown [P6]

Q10. Patient: Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes

- No, because: _____ [Q13] [P6]
- Unknown [Q13] [P6]

Q11. Patient: When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q13] [P6]
- Never [Q13] [P6]
- Unknown [Q13] [P6]

Q12. Patient: When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q13] [P6]
- More than 5 years ago [Q13] [P6]
- Never [Q13] [P6]
- Unknown [Q13]

Q13. Patient: Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P6]
- Unknown [P6]

Q14. Patient: How would you say your health is, in general?(1)

- Excellent
- Very good
- Good
- Fair
- Poor

Q15. Patient: How would you describe your health in the past year?

- Much better
- Somewhat better
- About the same
- Somewhat worse
- Much worse

Q16. Patient: How tall are you?

- Height: _____

Q17. Patient: How much do you weigh?

- Weight: _____ Nutritional Status, Adult  CCG

Q18. Patient: Do you get regular exercise?

- Yes Exercise  CCG
- No
- Unknown

Functional Status

Q1. Patient: Does your health limit your activities of daily living, such as bathing, dressing, eating, or moving around?

- Yes Activities of Daily Living  CCG Safety  CCG
- No
- Unknown Activities of Daily Living  CCG Safety  CCG

Q2. Patient: Does your health limit your instrumental activities of daily living, such as doing housework, taking your medicine, managing money, getting to and from places, or shopping?

- Yes Instrumental Activities of Daily Living  CCG
- No
- Unknown Instrumental Activities of Daily Living  CCG

Q3. Patient: Do you use any devices to assist you for care or safety?

- Yes Assistive Devices  CCG Safety  CCG

- No
- Unknown Assistive Devices [CCG](#) [Safety](#) [CCG](#)

Q4. Patient: Do you have any problems with sleeping?

- Yes Sleep [CCG](#)
- No
- Unknown Sleep [CCG](#)

Q5. Patient: Do you have uncontrolled pain?

- Yes Pain Assessment [CCG](#)
- No
- Unknown Pain Assessment [CCG](#)

Patient Care

Q1. Patient: What conditions would you like help with managing? (Check all that apply.)

- Anemia Anemia - Self-Care [CCG](#)
- Anxiety Anxiety - Self-Care [CCG](#)
- Arthritis Arthritis - Self-Care [CCG](#)
- Asthma Asthma, Adult - Self-Care [CCG](#)
- Atrial fibrillation Atrial Fibrillation - Self-Care [CCG](#)
- Attention-deficit hyperactivity disorder (ADHD) Attention-Deficit Hyperactivity Disorder, Adult - Self-Care [CCG](#)
- Back pain Back Pain - Self-Care [CCG](#)
- Bipolar disorder Bipolar Disorder - Self-Care [CCG](#)
- Cancer Cancer - Self-Care [CCG](#)
- Cardiac congenital defects Cardiac Congenital Defects - Self-Care [CCG](#)
- Chronic obstructive pulmonary disease (COPD) Chronic Obstructive Pulmonary Disease - Self-Care [CCG](#)
- Coronary artery disease Coronary Artery Disease - Self-Care [CCG](#)
- Cystic fibrosis Cystic Fibrosis, Adult - Self-Care [CCG](#)
- Dementia Dementia - Self-Care [CCG](#)
- Depression Depression - Self-Care [CCG](#)
- Diabetes Diabetes, Adult - Self-Care [CCG](#)
- End-of-life care End-of-Life Care - Self-Care [CCG](#)
- Heart failure Heart Failure - Self-Care [CCG](#)
- High blood pressure Hypertension - Self-Care [CCG](#)
- High cholesterol High Cholesterol - Self-Care [CCG](#)
- High-risk pregnancy High-Risk Pregnancy - Self-Care [CCG](#)
- HIV/AIDS HIV/AIDS - Self-Care [CCG](#)
- Inflammatory bowel disease Inflammatory Bowel Disease - Self-Care [CCG](#)
- Kidney failure Kidney Disease - Self-Care [CCG](#)
- Liver disease Liver Disease - Self-Care [CCG](#)
- Multiple sclerosis Multiple Sclerosis - Self-Care [CCG](#)
- Neonatal care Neonatal Care - Self-Care [CCG](#)
- Obesity Obesity, Adult - Self-Care [CCG](#)
- Organ transplant Organ Transplant - Self-Care [CCG](#)
- Osteoporosis Osteoporosis - Self-Care [CCG](#)
- Palliative care Palliative Care - Self-Care [CCG](#)
- Parkinson disease Parkinson Disease - Self-Care [CCG](#)
- Pregnancy Pregnancy - Self-Care [CCG](#)
- Schizophrenia Schizophrenia - Self-Care [CCG](#)
- Seizure disorder Seizure Disorders - Self-Care [CCG](#)

- Sickle cell disease Sickle Cell Disease, Adult - Self-Care  CCG
- Sleep apnea Sleep Apnea - Self-Care  CCG
- Spinal cord injury Spinal Cord Injury - Self-Care  CCG
- Stroke Stroke - Self-Care  CCG
- Substance use disorder Substance-Related Disorders - Self-Care  CCG
- Traumatic brain injury Traumatic Brain Injury - Self-Care  CCG
- Other, specify: _____
- None

Q2. Patient: What complementary or alternative therapies do you use? (Check all that apply.)

- Acupressure [P7]
- Acupuncture [P7]
- Art or music therapy [P7]
- Ayurveda [P7]
- Bioelectromagnetic therapy (eg, magnetic, pulsed, or current fields) [P7]
- Biofield therapy (eg, Reiki, qigong, therapeutic touch) [P7]
- Chiropractic or osteopathic manipulation [P7]
- Cupping [P7]
- Dietary supplements, specify: _____ [P7]
- Herbs, specify: _____ [P7]
- Homeopathy, specify: _____ [P7]
- Hypnotherapy [P7]
- Magnet therapy [P7]
- Massage, progressive relaxation, or guided imagery [P7]
- Meditation or prayer [P7]
- Religious or cultural healers [P7]
- Vitamins or minerals, specify: _____ [P7]
- Yoga, Pilates, or tai chi [P7]
- Other, specify: _____ [P7]
- None
- Unknown [P7]

Medication

Q1. Patient: What are the names, doses, routes, and frequencies of all the medicines you take? (Include nonprescription medicines.)

- No medicine taken
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
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- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Unknown [P8]

Q2. Patient: Do you know what each of these medicines is for?

- Yes

- No [P8]
- Unknown [P8]
- Not applicable

Q3. Patient: In the last months, have you had any bothersome side effects, reactions, or problems from your medicines? (Check all that apply.)

- Allergic reactions [Q4] [P8]
- Bladder changes (urinating more or less) [Q4] [P8]
- Blood sugar changes [Q4] [P8]
- Bowel changes such as loose stools (diarrhea) or hard stools (constipation) [Q4] [P8]
- Dizziness or tiredness that affects daily activity [Q4] [P8]
- Upset stomach (nausea) or throwing up (vomiting) [Q4] [P8]
- Vital sign changes (heart rate, blood pressure, respirations, temperature, pain) [Q4] [P8]
- Weight changes [Q4] [P8]
- Other, specify: _____ [Q4] [P8]
- Unknown [Q4] [P8]
- Not applicable

Q4. Patient: Have you talked to your doctor about these symptoms?

- Yes, and the doctor said: _____
- No [P8]
- Unknown [P8]
- Not applicable

Q5. Patient: Are you taking any new medicines prescribed by the doctor?

- Yes, specify: _____ [P9]
- No, I do not have any new medicine.
- No, I haven't started taking the new medicine. Medication Adherence  CCG
- Unknown Medication Adherence  CCG

Q6. Patient: Are you taking any new medicines that you got without a prescription (eg, over-the-counter medicines, vitamins, minerals, or herbal or dietary supplements)?

- Yes, specify: _____ [P9]
- No
- Unknown

Q7. Patient: Are there any medicines that you are no longer taking?

- Yes, specify: _____ [P9]
- No
- Unknown

Q8. Patient: Are there any medicines that you are taking more or less of?

- Yes, specify: _____ [P9]
- No
- Unknown

Q9. Patient: Do you have a list of your current medicine information?

- Yes
- No [P9]
- Unknown

Q10. Patient: Where do you keep your medicine?

- Bathroom cabinet [P10]
- Bedside tables [P10]
- Kitchen cabinet [P10]
- Kitchen counter [P10]
- Pillbox [P10]
- Refrigerator [P10]
- Other, specify: _____ [P10]
- Unknown [P10]
- Not applicable

Q11. Patient: How much of your medicine do you have before needing a refill?

- 3 months or more
- 1 to 3 months
- 2 to 4 weeks [P11]
- 2 weeks or less [P11]
- Unknown [P11]
- Not applicable

Q12. Patient: Do you have fewer than 2 refills for any of your medicine? (Look at the prescription label for the information on "number of refills before" date.)

- Yes [P11]
- No
- Unknown [P8]
- Not applicable

Q13. Patient: Do you take all your medicines as ordered?

- Yes
- No [P8] Medication Adherence [CCG](#)
- Unknown [P8]
- Not applicable

Psychosocial Assessment

Q1. Patient: Do you currently smoke or use tobacco products? (Check all that apply.) **QM**

- Yes, I smoke. [Q2] [Q4] [Q5] [Q6]
- Yes, I use other tobacco product; specify: _____ [Q3] [Q4] [Q5] [Q6]
- No, I quit. [Q4] [Q7]
- No, I have never smoked or used other tobacco products.
- Unknown

Q2. Patient: How much do you smoke?

- 2 packs a day or more [P12]
- More than 1 but less than 2 packs a day [P12]
- 1 pack a day [P12]
- Less than 1 pack a day [P12]
- Unknown
- Not applicable

Q3. Patient: How often do you use other tobacco products?

- 3 or more times a day [P12]
- 1 or 2 times a day [P12]
- A few times a week [P12]
- A few times a month [P12]
- A few times a year [P12]
- Unknown [Q6]
- Not applicable

Q4. Patient: How old were you when you started smoking or using tobacco products?

- Specify age: _____
- Unknown

Q5. Patient: What triggers you to want to smoke or use tobacco? (Check all that apply.)

- After meals [P12]
- Alcohol use [P12]
- Being around people who are smoking [P12]
- Driving or talking on the phone [P12]
- Stress [P12]
- Other, specify: _____ [P12]
- Nothing [P12]
- Unknown [P12]
- Not applicable

Q6. Patient: Are you ready to stop smoking or using tobacco products? **QM**

- Yes [P12]
- No [P12] Readiness to Change  CCG
- Unknown [P12] Readiness to Change  CCG

Q7. Patient: When did you quit smoking or using tobacco products?

- More than 1 year ago [P13]
- Within the last year [P13]
- Unknown

Q8. Patient: Do you drink alcohol?

- Yes Alcohol Use  CCG
- No
- Unknown Alcohol Use  CCG

Q9. Patient: Do you misuse any substances (such as illegal drugs, or drugs not prescribed to you)?

- Yes, specify: _____ Substance Use  CCG
- No
- Unknown Substance Use  CCG

Q10. Case Manager: Has a psychosocial assessment been completed?

- Yes
- No Start of Care Psychosocial Assessment, Adult  CCG
- Unknown Start of Care Psychosocial Assessment, Adult  CCG

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.

Plan of Care

Barriers

- B1.** Health literacy barrier
- B2.** Linguistic barrier
- B3.** Hearing impairment
- B4.** Vision impairment

Problems

P1. Language barrier present

- **Goal:** Language barrier resolved 
 - **Interventions:**
 - **ASSIST:** Identifying translation options and using the most practical and appropriate one(2)
 - **COORDINATE:** Call with patient and translation service and obtaining consent from patient to use translator(3)(4)
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **EDUCATE:** Ways to overcome language barriers(2)(5):
 - Speak slowly in normal tone.
 - Discuss one topic at a time.
 - Repeat and summarize often.
 - Use names instead of pronouns.
 - Use short, simple words and sentences.
 - Use interpretation or translation services.(6)
 - **COORDINATE:** Communication to providers to offer education materials in native language, whenever possible(6)(7)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
 - **COORDINATE:** Follow-up contact to evaluate language barrier status, referral status, and need for further assistance
 - **SEND:** What you need to know - Chronic condition management  (Spanish)

P2. Hearing evaluation needed

- **Goal:** Hearing evaluated and self-care understood^N
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation
 - **ASSIST:** Appointment scheduling for hearing evaluation
 - **EDUCATE:** When to get help for hearing problems (eg, infection, hearing problems get worse, social isolation, or depression)(11)(12)
 - **EDUCATE:** Hearing loss implications (eg, safety risks)(11)
 - **COORDINATE:** Audiologist referral, if indicated(11)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Care team meeting, if needed(10)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
 - **COORDINATE:** Follow-up contact to evaluate hearing status, referral status, and need for further assistance(14)(15)
 - **SEND:** What you need to know - Chronic condition management  (Spanish)

P3. Hearing impairment present

- **Goal:** Hearing maximized(16)^N
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation
 - **ASSIST:** Appointment scheduling for evaluation of hearing impairment(11)^N
 - **ASSIST:** Informing patient's providers of hearing loss and preferred alternative communication method
 - **ASSIST:** Identifying resources for hearing aids, phone lines, and other needed equipment and training
 - **ASSIST:** Utilizing a sign language interpreter trained for medical settings who can assist during medical appointments, if appropriate
 - **ASSIST:** Obtaining instructions in writing as much as possible(17)
 - **EDUCATE:** Ways to work around hearing loss(11)(17):
 - Hearing aid or assistive device^N
 - Writing or drawing pictures
 - Reading lips
 - Technical aids (eg, TTY phone service or closed-captioning)
 - Gestures or sign language
 - **EDUCATE:** Hearing aids, benefit, use, and care, if appropriate(18)
 - **EDUCATE:** Hearing loss implications (eg, safety risks)(18)^N
 - **EDUCATE:** Ways to prevent further hearing problems(11)(18):
 - Avoid places with loud persistent noises.
 - Keep environmental noises down when trying to listen.
 - Manage ear problems (eg, infection) quickly.
 - **EDUCATE:** Ways caregivers can help with communication(11)(19)(20):
 - Devote full attention when speaking or listening to patient, and avoid distracting environments (eg, those with competing noise).
 - Face patient when speaking, talk slowly, and do not cover mouth.
 - Ask patient to repeat back what they heard.
 - Avoid jumping from topic to topic, and speak in simple sentences.
 - Provide other ways to communicate (eg, offering paper for writing).
 - Promote use of hearing aids, as appropriate.(21)
 - **EDUCATE:** When to get help for hearing problems (eg, infection, hearing problems get worse, social isolation, or depression)(11)(21)
 - **EDUCATE: Resource:** Hearing Loss Association of America: www.hearingloss.org
 - **COORDINATE:** Otolaryngologist referral, if indicated(12)
 - **COORDINATE:** Audiologist referral, if indicated(11)
 - **COORDINATE:** Rehabilitation therapy referral, if indicated(22)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Care team meeting, if needed(10)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)

- **COORDINATE:** Follow-up contact to evaluate hearing status, referral status, and need for further assistance(14)(15)
- **SEND:** What you need to know - Chronic condition management  (Spanish)

P4. Vision impairment present

- **Goal:** Vision maximized(16)(23)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation
 - **ASSIST:** Appointment scheduling for evaluation of visual impairment
 - **ASSIST:** Informing patient's providers of vision loss and patient's preference for communication
 - **ASSIST:** Identifying resources for visual aids (glasses, contact lenses) and other needed equipment and training
 - **ASSIST:** Obtaining written materials in braille, if indicated
 - **EDUCATE:** Ways to work around vision loss(24):
 - Visual aids or assistive devices (glasses, contact lenses)
 - Alternative ways to perform activity
 - Voice options for electronics
 - Sighted companion
 - Cane and mobility training
 - **EDUCATE:** Implications of vision impairment(24):
 - Interference with ADL and increased risk for falls and other injuries
 - Impact on psychological, emotional, and social well-being
 - **EDUCATE:** Ways to prevent further vision problems:
 - Keep eyes healthy (nutrition support, washing hands before eye care, wearing protective eyewear).
 - Importance of evaluation for changes in vision
 - Manage eye problems (eg, infection) quickly.
 - **EDUCATE:** When to get help for vision problems (eg, infection, vision problems get worse, social isolation, or depression)(21)(24)
 - **EDUCATE: Resource:** American Foundation for the Blind: www.afb.org
 - **EDUCATE:** Routine eye examinations important
 - **COORDINATE:** Optometrist referral, if indicated
 - **COORDINATE:** Ophthalmologist referral, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated(24)
 - **COORDINATE:** Medical equipment and supplies, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Care team meeting, if needed(10)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
 - **COORDINATE:** Follow-up contact to evaluate vision status, referral status, and need for further assistance(14)(15)
 - **SEND:** What you need to know - Chronic condition management  (Spanish)

P5. Vision evaluation needed

- **Goal:** Vision evaluated and self-care understood(23)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation
 - **ASSIST:** Appointment scheduling with provider for visual evaluation
 - **EDUCATE:** When to get help for vision problems (eg, infection, vision problems get worse, social isolation, or depression)(21)(24)
 - **EDUCATE:** Vision loss implications (eg, safety risks)(16)
 - **COORDINATE:** Optometrist referral, if indicated
 - **COORDINATE:** Ophthalmologist referral, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Follow-up contact to evaluate vision status, referral status, and need for further assistance(14)(15)
 - **SEND:** What you need to know - Chronic condition management  (Spanish)

P6. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(25)(26)

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling
- **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
- **EDUCATE:** Purpose and benefits of vaccinations
- **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: www.cdc.gov/vaccines(27)
 - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(28)
 - Possible side effects or complications associated with getting vaccinated
- **EDUCATE:** Influenza vaccination annually(29):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
- **EDUCATE:** Pneumococcal vaccination potential side effects:
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.
 - Serious side effects are rare.
- **EDUCATE:** COVID-19 vaccination potential side effects(27):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
 - Serious side effects are rare.
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Vaccinations
- **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(14)(15)
- **SEND:** Coronavirus disease 2019 (COVID-19) vaccines 📎 (Spanish)
- **SEND:** Flu vaccine 📎 (Spanish)
- **SEND:** Pneumonia vaccine for adults 📎 (Spanish)
- **SEND:** Hepatitis A vaccine 📎 (Spanish)
- **SEND:** Hepatitis B vaccine 📎 (Spanish)

P7. Complementary, alternative, or integrative therapies evaluated for effect on treatment plan and outcomes

- **Goal:** Complementary, alternative, or integrative therapies will support positive treatment outcomes.(30)(31)(32)

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Identifying alternative treatments patient may be using
- **ASSIST:** Appointment scheduling with provider to evaluate participation in complementary, alternative, or integrative therapy with treatment plan
- **EDUCATE:** Talk to provider before starting any complementary, alternative, or integrative therapy.
- **EDUCATE:** Some complementary, alternative, or integrative therapies can interfere with treatment and outcomes.
- **EDUCATE:** Known benefits and risks of participation in complementary, alternative, or integrative therapy(33)
- **EDUCATE:** Select complementary, alternative, or integrative therapy providers who are appropriately licensed (if applicable), trained, and experienced.
- **EDUCATE:** Resource: National Center for Complementary and Integrative Health: www.nccih.nih.gov
- **COORDINATE:** Social worker referral, as needed(34)
- **COORDINATE:** Communication between care team members regarding use of complementary, alternative, or integrative therapies(10)
- **COORDINATE:** Follow-up contact to evaluate patient's status with referrals and treatment, potential for problems, and need for further assistance(14)(15)

P8. Medication information needed

- **Goal:** Medication information understood📎

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
- **ASSIST:** Creation of current and reconciled medication list
- **ASSIST:** Identifying activities that promote medication adherence
- **ASSIST:** Identifying medication support persons (eg, family, caregiver) to assist patient with managing medication(35)
- **EDUCATE: Reason for taking medication(36):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - What happens if medication is not taken as prescribed
- **EDUCATE:** Information on all medications(35)(36)(37)(38):
 - Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
 - Name of provider who ordered medication
 - Administration techniques
 - Side effects, related management, and when to contact provider
 - **Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - Common side effects include gastrointestinal changes, rash, dizziness, and headache.
 - Do not stop long-term medications abruptly due to risk of withdrawal symptoms.
 - Medication safety (eg, proper storage and disposal)(39)
 - Monitoring (eg, vital signs, laboratory checks) needed while taking medication
 - Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
 - Carry a list of medications at all times, particularly when visiting healthcare providers.
- **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)(40)
- **COORDINATE:** Communication between care team members and medication list reconciliation.(38)(41) See Medication Reconciliation Tool  SR for more information.
- **COORDINATE:** Obtaining medication organizer or reminder tools (eg, pillbox)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Social worker referral, as needed(34)
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(14)(15)
- **SEND:** Managing your medicines (Illustrated) (Spanish)

P9. Medication changes present

- **Goal:** Medication changes understood and medication taken as ordered
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of medication problems (clarification of dose or regimen)
 - **ASSIST:** Creation of current and reconciled medication list
 - **ASSIST:** Identifying activities that promote medication adherence
 - **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
 - **EDUCATE: Reason for taking medication(36):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - Risks of nonadherence
 - **EDUCATE:** Information on all medications(35)(36)(37)(38):
 - Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
 - Name of provider who ordered medication
 - Administration techniques
 - Side effects, related management, and when to contact provider

- **Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
- Common side effects include gastrointestinal changes, rash, dizziness, and headache.
- Do not stop long-term medications abruptly due to risk of withdrawal symptoms.
- Medication safety (eg, proper storage and disposal)(39)
- Monitoring (eg, vital signs, laboratory checks) needed while taking medication
- Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
- Carry a list of medications at all times, particularly when visiting healthcare providers.
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Communication between care team members and medication list reconciliation.(38)(41) See Medication Reconciliation Tool  SR for more information.
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)(40)
- **COORDINATE:** Follow-up contact for evaluation of medication understanding, referral status, and need for further assistance(14)(15)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P10. Medication safety information needed

- **Goal:** Medication safety understood and medication safety implemented(42)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with primary care provider when another provider (eg, emergency provider or other specialist) prescribes new medication
 - **EDUCATE:** Medication safety instructions(38):
 - Discard outdated medication.
 - Do not share prescription medication.
 - Store out of reach of children and in childproof containers.
 - Fill all prescriptions at the same pharmacy.
 - Ensure proper storage (in original containers and with other needs such as refrigeration or protection from sunlight).
 - Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
 - **EDUCATE:** Administration techniques
 - **EDUCATE:** De-prescribing or cessation of medications safely (eg, pharmacy-supported, provider monitoring planned) (43)
 - **COORDINATE:** Communication with pharmacy for medication interaction review
 - **COORDINATE:** Communication between care team members and medication list reconciliation.(41) See Medication Reconciliation Tool  SR for more information.
 - **COORDINATE:** Follow-up contact for evaluation of medication understanding, referral status, and need for further assistance(14)(15)
 - **SEND:** Managing your medicines (Illustrated)  (Spanish)

P11. Medication refills needed

- **Goal:** Medications refilled
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **ASSIST:** Obtaining refills in lieu of an appointment
 - **ASSIST:** Identifying resources to pay for medications, if needed
 - **ASSIST:** Working with pharmacy for filling prescriptions (eg, changing pills to liquids, identifying financial support, delivery)(37)
 - **EDUCATE:** Importance of getting medications refilled prior to running out (1 to 2 weeks before)
 - **EDUCATE:** Pharmacy assistance programs, such as:
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>

- NeedyMeds: www.needymeds.org
- Individual company websites
- **COORDINATE:** Communication with pharmacy (eg, home delivery, change in medication form, help with drug-drug or food-drug problems)
- **COORDINATE:** Pharmacy medication support, if indicated
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(14)(15)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P12. Tobacco use present

- **Goal:** Tobacco abstinence achieved(44) **NNN**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation for nicotine replacement therapy (NRT) or other medications to help patient quit using tobacco
 - **ASSIST:** Developing a plan to quit(45):
 - Setting quit date(46)
 - Identifying reasons to quit (eg, add years to your life, for someone you love, save money, feel better)
 - Identifying techniques to quit (ie, "cold turkey," using nicotine replacement, or gradual withdrawal)(47)
 - Identifying coping mechanisms
 - **ASSIST:** Identifying resources to help tobacco cessation
 - **EDUCATE:** Impact of tobacco on life(48):
 - Current symptoms caused by tobacco use
 - Costs of tobacco use
 - Impact of tobacco use on home environment
 - Risks of continued smoking on health (eg, cancer, stroke, gum disease)
 - **EDUCATE:** Benefits of tobacco cessation:
 - Within minutes to days - lower blood pressure, lower carbon monoxide levels, better stamina, and improved smell and taste
 - Within 2 to 4 weeks - less risk of respiratory infection
 - Within 1 to 3 months - improved lung function and circulation
 - Within 1 year - 50% reduction in risk for myocardial infarction(49)
 - Within 5 to 15 years - cardiovascular risk equal to that of someone who has never smoked
 - Within 10 years - lung cancer risk cut in half
 - **EDUCATE:** Ways to stop smoking or using tobacco(48):
 - Get support (support group, counseling, family buy-in, written self-help materials).(50) **NNNNN**
 - Talk to providers for help.
 - Stress management (eg, relaxation and coping techniques)(46)
 - Nicotine replacement therapy (eg, patch, gum) or medications that may help(A) **N**
 - Healthy lifestyle (good nutrition, activity)(46)(51) **N**
 - **EDUCATE:** Withdrawal symptoms that may be experienced (eg, trouble sleeping, replacing smoking urge with eating)
 - **EDUCATE: Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
 - **COORDINATE:** Motivational counseling if patient not ready to quit(52)
 - **COORDINATE:** Tobacco use treatment, as needed
 - **COORDINATE:** Participation in stop-smoking programs (eg, American Cancer Society, American Lung Association)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
 - **COORDINATE:** Follow-up contact to monitor for withdrawal and evaluate tobacco use, referral status, and need for further assistance(14)(15)
 - **SEND:** How to stop smoking  (Spanish)
 - **Goal:** Tobacco cessation medication treatment initiated and maintained(53) **NN**
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Medication is used to reduce withdrawal feeling during smoking cessation.
 - Bupropion sustained-release and varenicline are first-line treatment, either alone or in combination.
 - Bupropion is recommended for 7 to 12 weeks. Continued treatment may be needed.
 - Varenicline can be used as maintenance therapy to prevent relapse.(54)

- **EDUCATE: Call provider immediately:** Suicidal ideation, behavioral changes, or seizures(55)
- **EDUCATE: Side effects:** Insomnia, tachycardia
- **EDUCATE: Considerations:** Avoid bupropion in patients with seizure disorders.(53)
- **SEND:** How to stop smoking 📄 (Spanish)
- **Goal:** Nicotine replacement therapy (NRT) initiated and maintained(53)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** NRT is used to reduce withdrawal feeling during smoking cessation.
 - Dosage is individualized based on need and response of patient.
 - If using patch, apply one new patch every 24 hours on skin that is dry, clean, and hairless. Rotate skin sites. Do not cut patch, and do not use more than one patch at a time.
 - If using lozenge or gum, do not eat or drink for 15 minutes before or while utilizing.
 - **EDUCATE: Call provider immediately:** Nicotine toxicity (eg, nausea, abdominal pain, vomiting, diarrhea, sweating, disturbed vision or hearing, palpitations, altered respirations)
 - **EDUCATE: Side effects:** Anxiety, sleep disorders, depression, myalgia
 - Patch may cause redness or swelling of the skin around the patch that does not go away.
 - **EDUCATE: Considerations:** Avoid nicotine use in immediate post-MI period or in presence of serious arrhythmias or severe or worsening angina.
 - **SEND:** How to stop smoking 📄 (Spanish)

P13. Tobacco use in patient history

- **Goal:** Tobacco abstinence maintained(48)(56)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of improved or stabilized health status, for reinforcement of abstinence from tobacco
 - **EDUCATE:** How to keep environment tobacco free
 - **EDUCATE:** Encourage continued abstinence from smoking and review tobacco cessation strategies.
 - **EDUCATE: Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
 - **COORDINATE:** Tobacco use treatment, as needed
 - **COORDINATE:** Support group referral, as needed
 - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
 - **COORDINATE:** Follow-up contact to evaluate tobacco use, referral status, and need for further assistance(14)(15)

Evidence Summary

Quality standards: Standards indicate that a needs assessment should be completed for every patient to identify and resolve potential communication barriers, including vision impairments, hearing impairments, and languages spoken other than English. Programs should have a process for following up with patients on the status of referrals.(57)

National guideline: The U.S. Preventive Services Task Force recognizes that age-related sensorineural hearing loss can adversely affect quality of life and functional abilities and is associated with increased risk of falls, hospitalizations, social isolation, and cognitive decline; however, there is insufficient evidence to assess the balance of benefits and harms of screening patients without symptoms of hearing loss.(58)

Evidence: In a study of 99 adults (age 62 to 82 years), self-reported overall quality of life was significantly improved at 18 months after hearing aid fitting.(59)

Evidence: Multiple studies conclude that hearing loss contributes to social isolation and mood disorders and is associated with increased rates of cognitive impairment.(11)(60)(61)

National guideline: The U.S. Preventive Services Task Force recognizes that vision screening of older adults is important for the detection of age-related vision changes (eg, cataracts, refractive errors, adjustment to light intensity changes, distinguishing color, glare tolerance); however, there is insufficient evidence that vision screening in primary care for patients without symptoms of vision problems results in improved functional outcomes (eg, fewer falls).(62)

Quality standards: The American Case Management Association's Transitions of Care Standards indicate that medications should be reconciled at each care transition and an evaluation should be done of the patient/family/caregiver's understanding of health status and self-care.(63)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(64)

Evidence: A prospective cohort study of 2655 hospitalized patients after discharge found that 24% of hospital medication changes were not followed, 30% of changed doses were filled with the incorrect dose, 27% of new medications were not filled, and 12% of discontinued medications were filled. Risk factors for medication change problems included increased out-of-pocket costs, not having medications filled before hospitalization, and discharge to a long-term care facility.(65)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, substance use). Programs should have a process for following up with patients on the status of referrals.(57)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) recommends that clinicians ask all adults about tobacco use, advise them to stop using tobacco, and provide behavioral interventions and FDA-approved pharmacotherapy for cessation to nonpregnant adults who use tobacco. The USPSTF recommends that clinicians ask all pregnant persons about tobacco use, advise them to stop using tobacco, and provide behavioral interventions for cessation to those who use tobacco.(66)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) concludes that there is insufficient evidence to determine the balance of benefits and harms of primary care interventions for tobacco cessation among school-age children and adolescents who already smoke because of a lack of adequately powered studies on behavioral counseling interventions and a lack of studies on medications. Clinical judgment should be used to decide how to best help youth who use tobacco.(67)

Evidence: A systematic review of 33 trials with 21,600 participants found that incentives (eg, money, vouchers) improved smoking cessation rates at long-term follow-up (at least 6 months) in mixed-population studies. Ten trials of 2571 pregnant women found that women in the incentive-rewards group were more likely to stop smoking.(68)

Evidence: A systematic review of 47 studies with more than 18,000 people showed improved smoking cessation with increased therapeutic support and nicotine replacement therapy (NRT) as compared with NRT alone.(50)

Evidence: A review of 43 studies with more than 28,000 people found with moderate certainty that when no other support is available, written self-help materials help more people to stop smoking compared with getting no help at all.(69)

Evidence: A meta-analysis of 10 randomized studies of 3655 participants using nicotine replacement therapy found that behavioral support focusing on adherence to smoking cessation medications can modestly improve adherence.(70)

Evidence: A network meta-analysis of 33 systematic reviews with 250,563 participants found that behavioral support for smoking cessation can increase quit rates at 6 months or longer, with the strongest benefit for the provision of any type of counseling and guaranteed financial incentives.(71)

Evidence: A systematic review of 136 trials of nicotine replacement therapy (NRT) with 64,640 people found that all forms of NRT made it more likely that a person's attempt to quit smoking would succeed. Their chances of stopping smoking increased by 50% to 60% with or without additional counseling. There is no evidence that NRT increases the risk of heart attacks. In pregnant women, evidence suggests that NRT can increase the chance of quitting at the time of delivery.(72)

Evidence: A systematic review of 24 trials with 7279 participants found no evidence that adding exercise to smoking cessation support improves abstinence compared with support alone, but the evidence is insufficient to assess whether there is a modest benefit.(73)

Evidence: A systematic review of 51 studies including 22,000 people who smoked tobacco found that cessation was more likely if smokers cut down their intake when using stop-smoking medication (eg, varenicline) or fast-acting nicotine replacement therapy vs quitting all at once.(47)

Specialty Society: The American Thoracic Society recommendations for tobacco-dependent adults include using varenicline rather than a nicotine patch, using varenicline rather than bupropion, using varenicline rather than a nicotine patch in adults with a comorbid psychiatric condition, initiating varenicline in adults even if they are unready to quit, and using controller therapy for an extended treatment duration greater than 12 weeks.(54)

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Footnotes

[A] Primary pharmacotherapy for tobacco dependence includes varenicline, bupropion, nicotine gum, nicotine inhaler, nicotine nasal spray, and nicotine patch. Nicotine replacement therapies (NRTs) should be used with caution for patients in the 2 weeks following myocardial infarction and for those with life-threatening arrhythmias or severe angina.(44)(66) [A in Context Link 1]

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